Fax cover sheet

Sunridge Valley Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Owen Prescott, LCSW
Phone: 555-0181
Date: 10/15/2026 08:40
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

34F asthma exacerbation, resolved. Independent, walking halls.
Hoping for d/c to recuperative care 10/15 PM. Please call with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Sunridge Valley Medical Center (fictional)
Dept/Floor: 4 East Medical
SW/CM/RN: Owen Prescott, LCSW
Phone/Pager #: 555-0181
Email: oprescott@sunridge.example
Nursing Station #: 555-0182
Authorized by: Dr. M. Castellano
Phone #:

Patient Information

Patient Name: Angela Doe MRN: SV-2093516 Date of Birth (dd/mm/yyy): 02/03/1992
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Black or African American
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Acute asthma exacerbation
Admit Date: 10/12/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/12/2026 Results: CXR - hyperinflation, no infiltrate

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	X Yes* No	Active Infection	Yes* X No
*Explain 2L nasal cannula		*Explain	
Self Administer All Meds	X Yes No*	Ambulatory	X Yes No
Continent of Bowel & Bladder	X Yes No	Assistive Device	Yes* X No
Colostomy/Ileostomy	Yes X No	*Explain	
Foley Catheter	Yes X No	Hospice	Yes* X No
Diabetes	Yes X No	*Explain	
Insulin	Yes X No	Car	Yes X No
Diabetes related Oral Meds	Yes X No	Service Animal	Yes X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

Length of Stay Authorized in Recupertive Care: 10 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Angela Doe

DOB: 02/03/1992

Member ID: OCH-55841177

Referring Provider

Facility: Sunridge Valley Medical Center (fictional)

Clinical

Primary Diagnosis: Asthma, moderate persistent, with acute exacerbation

ICD-10: J45.41

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/15/2026

Requested Days: 10

Admit Date: 10/12/2026

Clinical summary: asthma exacerbation resolved, room air, independent ADLs.

Plan Use Only

Status: Pending review

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Patient face sheet

Sunridge Valley Medical Center (fictional) - PATIENT FACE SHEET

Patient: Angela Doe
MRN: SV-2093516
Account #: 99093516
DOB: 02/03/1992
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/12/2026 03:10 (ED arrival 10/11/2026 23:45)
Admit Type: Inpatient
Unit/Bed: 4E-421
Attending: Miguel Castellano, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical

HISTORY AND PHYSICAL

Date: 10/12/2026

CC

Wheezing and shortness of breath x 2 days

HPI

34 y/o F with moderate persistent asthma, unhoused x 1 yr (sleeps in her car until it was towed 3 weeks ago, now outdoors), presents with wheezing and dyspnea after wildfire smoke exposure and running out of her controller inhaler. In ED SpO2 90% RA, given continuous albuterol nebs, IV methylprednisolone, magnesium. Placed on 2L NC.
No prior intubations. No ICU admissions.

PMH

- Asthma since childhood
- Seasonal allergic rhinitis

Meds PTA

Albuterol inhaler PRN (ran out); fluticasone-salmeterol (ran out 1 month ago)

Allergies

Penicillin (rash)

Social

Never smoker. Rare EtOH. Denies drug use.

Exam

T 98.6F HR 112 BP 132/84 RR 26 SpO2 93% 2L NC. Diffuse expiratory wheezes. Speaking in short sentences. AOx4.

Assessment/Plan

1. Acute asthma exacerbation - albuterol/ipratropium nebs q4h, methylprednisolone 40 mg IV q12h then prednisone taper, O2 to SpO2 >= 92%
2. Dispo - SW consult (unhoused)

Signed: Miguel Castellano, MD 10/12/2026 05:20

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#4

Date: 10/15/2026 07:30

S: Breathing back to baseline. No nighttime symptoms. Walking halls without dyspnea.

O: T 98.2F HR 78 BP 118/72 RR 16 SpO2 96% RA. Clear to auscultation, no wheezes. AOx4. Peak flow 380 L/min (personal best approx 400).

A/P:

1. Asthma exacerbation - resolved. Off nebulizers since 10/13. On room air since 10/13 10:00. Prednisone 40 mg PO daily, last dose 10/17. Restart fluticasone-salmeterol BID; albuterol MDI PRN. No antibiotics (no infection).
2. Dispo - medically stable for d/c to recuperative care today. Pulmonology f/u in 2 wks.

Signed: Miguel Castellano, MD

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Respiratory therapy note

RESPIRATORY THERAPY NOTE

Date: 10/15/2026 09:00
O2 weaned off 10/13 10:00; on room air since. Ambulatory oximetry 10/14: rest SpO2 97% RA, after walking 250 ft SpO2 96% RA.
Nebulizer treatments discontinued 10/13. No breathing treatments needed in last 48 hours other than her own MDI PRN (used once 10/14).
Inhaler technique taught with spacer; pt demonstrated correct technique with teach-back.
Asthma action plan reviewed.
No home O2 indicated.
Signed: D. Ferreira, RRT

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Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 10/14/2026 13:00

Bed mobility: independent

Transfers: independent

Gait: 250 ft independent, no assistive device, steady, no rest break needed

Stairs: 14 steps with rail, independent

Balance: good

Assessment

At baseline. No skilled PT needs. No DME needed.

Signed: R. Albescu, PT

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Nursing shift note

NURSING SHIFT NOTE

Date: 10/15/2026 06:00

Pt AOx4. Independent with all ADLs. Showered independently 10/14 PM. Continent of bowel and bladder, toilets independently.

Self-administers prednisone and inhalers, teach-back done.

Calm and cooperative. No behavioral concerns this admission.

Isolation status: none (standard precautions).

Room air, SpO2 96%. Afebrile.

Signed: T. Nakashima, RN

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Social work note

SOCIAL WORK NOTE

Date: 10/14/2026 15:30

Pt engaged, AOx4. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history.

No conservatorship. Not enrolled in FSP.

Unhoused x 1 yr; car towed, belongings with a friend. Agreeable to recuperative care and housing navigation referral.

Signed: Owen Prescott, LCSW

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Labs, imaging and vitals

LABS

10/12 | WBC 11.8 H | Eos 6% | Hgb 13.2 | Plt 265 | Na 138 | K 3.6 | Cr 0.7 | Glucose 142
10/14 | WBC 10.9 | K 3.9 | Glucose 118
10/12 Respiratory viral panel (incl. SARS-CoV-2 PCR): Negative
10/12 MRSA nares PCR: Negative
10/15 05:50 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/12 CXR PA/lat: Hyperinflation. No infiltrate, no effusion, no pneumothorax.

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/14 08:00	98.4	82	120/74	16	97%	RA
10/14 14:00	98.6	80	116/70	16	96%	RA
10/14 20:00	98.2	76	118/72	14	97%	RA
10/15 02:00	98.0	74	114/70	14	96%	RA
10/15 07:30	98.2	78	118/72	16	96%	RA

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Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Angela Doe MRN: SV-2093516

- Prednisone 40 mg PO daily through 10/17/2026 (2 doses remaining)
- Fluticasone-salmeterol 250/50 1 inhalation BID
- Albuterol HFA 2 puffs q4h PRN wheeze, with spacer
- Cetirizine 10 mg PO daily

No IV medications. No home O2. No antibiotics.

Patient able to self-administer all medications: Yes

Meds-to-beds: 30-day supply of all medications dispensed at discharge

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